

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at medium risk for preeclampsia and medium risk for macrosomia. Key findings include an elevated systolic blood pressure (142 mmHg) and proteinuria (dipstick 1), which require clinical attention given the medium preeclampsia risk tier. The patient also has an elevated pre-pregnancy BMI (42.4) and an elevated fasting glucose (99 mg/dL), contributing to the macrosomia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, 32 weeks gestation, has a medium preeclampsia risk tier (score: 0.6395) and medium macrosomia risk tier (score: 0.5944). Key vitals include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. Fasting glucose is 99 mg/dL, with a pre-pregnancy BMI of 42.4. No severe preeclampsia symptoms reported. Clinical assessment is recommended for potential preeclampsia and further evaluation of glucose levels.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

